

[SYNTHETIC SAMPLE] Fictional record generated for the DELPHi prototype demo. Names, institutions, and statements are invented; the layout imitates a generic field-medical format. Not a real document and not derived from any real record.

[CONFERENCE REFERENCE] The meeting named above is a real, publicly announced scientific congress and is used here only as the setting of this fictional record. No session content, abstract, poster, or presented data from that meeting is reproduced or paraphrased; every person and conversation below is invented.

Congress Attendance Report — American Epilepsy Society Annual Meeting (AES) 2024

Meeting period: December 2024 | Author: Tom Alvarez

Scope: unsolicited conversations held on-site. Session content and presented data are deliberately not reproduced here.

Conversation — Yannick Adeyemi-Cole, MD (Willowmere Neuroscience Center)

The conversation happened while waiting for session room to clear. Monitoring intervals during titration are set by clinic routine here rather than by protocol. Talked through the pediatric-to-adult transfers in general terms, without any specific case.

Conversation — Alastair Harkness, MD (Quarry Hill Epilepsy Center)

We spoke standing near the exhibit aisles, briefly. The appeals process remains slowest step, which came up as general frustration. Described a preference for slower titration in general, unrelated to any particular patient. Left it at that, with following quarter to be scheduled.

Conversation — Hamish Ellingham, MD (Quarry Hill Epilepsy Center)

A short exchange over coffee queue. Described how the older half of the panel is triaged now compared with before. Onboarding one of the nurse practitioners is the current preoccupation here. Titration habits came up in general terms; no specifics were discussed and none were offered.

Conversation — Eamon Quintrell, MD (Fox Hollow Neurology Partners)

Introduced by a colleague from same region and talked for a few minutes. Titration scheduling is handled by nursing staff, which practice finds workable. Asked whether I had seen a case series someone forwarded; I had not, and offered nothing in return. Clinic throughput has stretched out since the summer, which shapes how much time there is for anything else.

Conversation — Silvio Fitzgibbon, MD (Thornfield Regional Epilepsy Program)

Approached me rather than other way around, which is worth noting. Spent some time on the pediatric-to-adult transfers and how workup decisions get sequenced. Asked whether I had seen a review that circulated on the service; I had not, and offered nothing in return.

Conversation — Josefina Dellacroix, MD (Sandhill Neuroscience Center)

Introduced by colleague from the same region and talked for few minutes. Mostly small talk about program and venue. Described how the refractory group is triaged now compared with before.

Conversation — Kenji Cavanaugh, MD (Highmoor Health Neurology)

Approached me rather than the other way around, which is worth noting. Most of the discussion was about the new charting templates, which is still being sorted out. Talked through the pediatric-to-adult transfers in general terms, without any specific case. Closed there — no specific request.

Conversation — Alice Nguyen-Barr, MD (Old Dominion Neurology Group)

Stopped to talk on way out of a session block, with no agenda on either side. Closed there — no product discussion. Referenced session summary from colleague in passing. Talked through pediatric-to-adult transfers in general terms, without any specific case.

Conversation — Odalys Thistlewood, MD (Silver Creek Medical Group)

Approached me rather than the other way around, which is worth noting. No specific request this time. Asked about safety and confirmed nothing to report. Titration habits came up in general terms; no specifics were discussed and none were offered. Formulary questions varies by payer, which came up as a general frustration.

Conversation — Hadley Wycliffe, MD (Northbrook Neurology Institute)

A short exchange over the coffee queue. Closed there — no materials requested. Referenced guideline update discussed at journal club in passing. Spent some time on the pediatric-to-adult transfers and how workup decisions get sequenced.

Conversation — Rosalind Voskuijen, MD (Willowmere Neuroscience Center)

We spoke standing near exhibit aisles, briefly. Compared notes on which sessions were worth the time. Coverage paperwork is handled by one person here, which came up as general frustration. Nothing that needed follow-up. Safety items asked about and confirmed none.

Conversation — Mairead Barrowman, MD (Foxglove Health Neurology)

We spoke standing near exhibit aisles, briefly. Mostly small talk about program and venue. Referenced a review that circulated on service in passing.

Follow-ups have been logged in the interaction system.